

PRODUCT LITERATURE

CALAC TABLET 300 MG

Each tablets contains

Calcium Lactate BP 300.00 mg

Preservatives

Sodium Benzoate BP 0.05%w/w

Product Description

Round, white coloured tablet.

Pharmacodynamics / Pharmacokinetics

N/A

Indication:

Health supplement

Recommended Dose:

Adults: 1 tablet, 3 times a day after food.

Route of Administration

Oral

Contraindications:

Except under special circumstances, these medications should not be used when the following medical problems exist:

- » Hypercalcemia, primary or secondary or
- » Hypercalciuria or
- » Renal calculi, calcium (risk of exacerbation)
- » Sarcoidosis (may potentiate hypercalcemia)

Risk-benefit should be considered when the following medical problems exist

- » Dehydration or

Electrolyte imbalance, other (may increase risk of hypercalcemia)

Diarrhea or

Malabsorption, gastrointestinal, chronic (fecal excretion of calcium may be increased, although patients with chronic diarrhea or malabsorption commonly need calcium supplements)

- » Renal calculi, history of (risk of recurrent stone formation)

» Renal function impairment, chronic (may increase risk of hypercalcemia; however, calcium carbonate or calcium citrate may be used as a phosphate binder in renal failure; also, some patients with renal failure have symptomatic hypocalcemia and need cautious treatment with calcium salts)

Warnings and Precautions:

Pediatrics

Problems in pediatrics have not been documented with intake of normal daily recommended amounts.

Geriatrics

Problems in geriatrics have not been documented with intake of normal daily recommended amounts. With advancing age, intestinal calcium absorption decreases. Therefore, calcium requirements are increased in the elderly, and dosages of oral supplements may need to be adjusted accordingly. Impaired absorption may be due to low levels of active vitamin D metabolites.

Interactions with Other Medicament:

Alcohol or

Caffeine, usually more than 8 cups of coffee a day or

Tobacco (concurrent use of excessive amounts of these substances has been reported to decrease calcium absorption)

Antacids containing aluminum (concurrent use with calcium citrate may enhance aluminum absorption)

Calcitonin (concurrent use with calcium supplements may antagonize the effect of calcitonin in the treatment of hypercalcemia; however, when calcitonin is prescribed for osteoporosis or

Paget's disease of the bone, calcium intake should be generous to prevent hypocalcemia which might generate secondary hyperparathyroidism; calcium-containing preparations may be given 4 hours after calcitonin)

Calcium-channel blocking agents (concurrent use of these medications with calcium supplements in quantities sufficient to raise serum calcium concentrations above normal may reduce the response to verapamil and probably other calcium-channel blockers)

» Calcium-containing preparations, other or
Magnesium-containing preparations, oral (concurrent use with calcium supplements may increase serum calcium or magnesium concentrations in susceptible patients, mainly patients with impaired renal function, leading to hypercalcemia or hypermagnesemia, respectively)

» Cellulose sodium phosphate (concurrent use with calcium supplements may decrease effectiveness of cellulose sodium phosphate in preventing hypercalciuria)

Contraceptives, estrogen-containing, oral or

Estrogens (concurrent use with calcium supplements may increase calcium absorption, which is used to therapeutic advantage when estrogens are prescribed for the treatment of postmenopausal osteoporosis)

» Digitalis glycosides (concurrent use of parenteral calcium salts with digitalis glycosides may increase the risk of cardiac arrhythmias; therefore, when the parenteral administration of calcium salts to digitalized patients is deemed necessary, caution and close electrocardiographic [ECG] monitoring are recommended)

Diuretics, thiazide (concurrent use with large doses of calcium supplements may result in hypercalcemia because of reduced calcium excretion)

» Etidronate (concurrent use with calcium supplements may prevent absorption of etidronate; patients should be advised not to take etidronate within 2 hours of calcium supplements)

Fiber, found in bran, whole-grain breads and cereals or

Phytates, found in bran and whole-grain breads and cereals (concurrent use of large amounts of fiber or phytates, especially in patients being treated for hypocalcemia, with calcium supplements may reduce calcium absorption by formation of nonabsorbable complexes)

Fluoroquinolones (concurrent use with calcium carbonate may reduce absorption by chelation of fluoroquinolones, resulting in lower serum and urine concentrations of fluoroquinolones; therefore, concurrent use is not recommended)

» Gallium nitrate (concurrent use with calcium supplements may antagonize the effect of gallium nitrate)

Iron supplements (concurrent use with calcium carbonate and calcium phosphate will decrease the absorption of iron; iron supplements should not be taken within 1 or 2 hours of calcium carbonate or phosphate; however, when iron and calcium carbonate are present in multivitamin-with-minerals formulations, the absorption of iron is not significantly changed, possibly because the ascorbic acid in the formulation maintains the iron in the ferrous state, thus increasing its solubility and absorption)

» Magnesium sulfate, parenteral (parenteral calcium salts may neutralize effects of parenteral magnesium sulfate and should be readily available to counteract the potentially serious risk of magnesium intoxication; also, calcium sulfate may precipitate when a calcium salt is admixed with magnesium sulfate in the same intravenous solution, although commercial nutritional solutions are formulated to avoid precipitation; calcium and magnesium should be administered through separate intravenous lines if required in post-parathyroidectomy "hungry bones" syndrome or tetany associated with hypocalcemia and hypomagnesemia)

Milk or milk products (concurrent excessive and prolonged use with calcium supplements may result in the milk-alkali syndrome)

Neuromuscular blocking agents, except succinylcholine (concurrent use with parenteral calcium salts usually reverses the effects of nondepolarizing neuromuscular blocking agents; also, concurrent use with calcium salts has been reported to enhance or prolong the neuromuscular blocking action of tubocurarine)

» Phenytoin (concurrent use decreases the bioavailability of both phenytoin and calcium because of possible formation of a nonabsorbable complex; patients should be advised not to take calcium supplements within 1 to 3 hours of taking phenytoin; also, enteral feeding solutions containing calcium should not be administered through a nasogastric feeding tube together with phenytoin oral suspension; a 2-hour interval should elapse between the administration of the feeding solution and of the phenytoin)

Potassium phosphates or

Potassium and sodium phosphates (concurrent use with calcium supplements may increase potential for deposition of calcium in soft tissues if serum ionized calcium is high)

Sodium bicarbonate (concurrent and prolonged use with calcium supplements may result in milk-alkali syndrome)

Sodium fluoride (concurrent use with calcium supplements may cause the calcium ions to complex with fluoride and inhibit absorption of both fluoride and calcium; however, if sodium fluoride is used with calcium supplements to treat osteoporosis, a one- to two-hour interval should elapse between doses)

»Tetracyclines, oral (concurrent use with calcium supplements may decrease absorption of tetracyclines because of possible formation of nonabsorbable complexes and increase in intragastric pH; patients should be advised not to take calcium supplements within 1 to 3 hours of taking tetracyclines)

Vitamin A (excessive intake, more than 7500 RE or 25,000 Units per day, of vitamin A may stimulate bone loss and counteract the effects of calcium supplementation and may cause hypercalcemia)

Vitamin D, especially calcifediol and calcitriol (concurrent use of large doses of vitamin D with calcium supplements may excessively increase intestinal absorption of calcium, increasing risk of chronic hypercalcemia in susceptible patients; however, it may be therapeutically advantageous in elderly and high-risk groups when it is necessary to prescribe vitamin D or its derivatives with calcium; careful monitoring of serum calcium concentrations is essential during long-term therapy)

Pregnancy/Reproduction

Pregnancy:

Studies have not been done in humans. However, problems have not been documented with intake of normal daily recommended amounts.

Studies have not been done in animals.

Second and third trimesters: Some studies have shown that calcium supplementation begun in the second trimester may be effective in lowering blood pressure in pregnant women with pregnancy-induced hypertension or pre-eclampsia, both of which may possibly be associated with increased calcium demand of the fetus during the last trimester.

During pregnancy, there is an increased need for calcium to calcify fetal bones and to increase the maternal skeletal mass in preparation for lactation. This need is normally met by enhanced intestinal absorption of calcium, increased vitamin D production, and a concurrent increase in calcitonin secretion, which prevents unwanted bone resorption in the maternal skeleton. The maternal parathyroid glands undergo hyperplasia, producing greater amounts of parathyroid hormone, which acts indirectly to increase intestinal absorption of calcium, reabsorption at the distal renal tubules, and bone calcium mobilization. However, the prescribing of calcium supplements during pregnancy may be necessary since standard prenatal vitamins along with normal intake of dairy products may not provide sufficient elemental calcium for the average pregnant woman.

FDA pregnancy categories not currently included in product labeling.

Breast-feeding

Problems in nursing babies have not been documented with intake of normal daily recommended amounts. Although some oral supplemental calcium may be distributed into breast milk, the concentration is not sufficient to produce an adverse effect in the neonate. It is not known whether calcium chloride or calcium gluconate is distributed into breast milk.

Side Effects

None

Symptoms and Treatment of Overdose

None

Packing

Packed in blister-packs of 3x10's, 10x10's and 50x10's tablets in a paper box.

Packed in HDPE bottle of 30's.

Pharmaceutical Precautions

Store below 30°C in a dry place, protected from direct light.

Keep away from reach of children.

Expiry Date

3 years from date of manufacture

Name and Address of Product Registration Holder/Manufacturer:

TERAPUTICS SDN. BHD. (590500-W)

Lot 10 & 11, PERDA Industrial Park,

Lorong IKS Simpang Ampat B,

14100 Simpang Ampat, S.P.S.,

Pulau Pinang, Malaysia

Name and Address of Distributor:

ZONTRON PHARMACEUTICALS SDN. BHD. (445695-T)

Lot 10 & 11, PERDA Industrial Park,

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Malaysian Drug Registration No. :

MAL19940205XC

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